

Discharge Summary USE BLACK INK ONLY

Patient Details Surname Sinclair Forename Margaret M / F/ (Female) Date of Birth 03/01/1947 NHS/ Hosp No. 654 128 7743 Address 45 Cedar Court (sheltered housing), Fleet, GU51 3RJ Tel No. 01252 334 5567 / NOK 07890 123456		Admission and GP Details Discharging Consultant Mr A. Chalmers Discharging Speciality/ Department T&O/Orthogeriatrics Method of Admission Emergency - ambulance (fall) Date of Admission 14/07/2026 Date of Discharge 25/07/2026 G.P. Details Dr L. Marsh, Fleet Road Surgery	
Diagnosis at Discharge Left displaced intracapsular fractured neck of femur. PMH: osteoporosis, hypertension, mild cognitive impairment.		Operations and Procedures Left cemented hemiarthroplasty (15/07/2026) under spinal anaesthetic, uncomplicated.	
Reason for Admission and Presenting Complaint(s) Found on the floor at home by carer after a mechanical fall, unable to weight-bear on the left leg; leg shortened and externally rotated. No head injury, no loss of consciousness.			
Clinical Narrative 79F with osteoporosis and mild cognitive impairment sustained a mechanical fall at home; radiographs confirmed a displaced left intracapsular NOF fracture. Admitted under the orthogeriatric joint-care pathway and underwent left cemented hemiarthroplasty within 36 hours, per NICE hip fracture standards. Post-operative course satisfactory, mobilised with physiotherapy from day 1, progressing from frame to elbow crutches. Wound healing well with staples in situ. Bone health assessed, bisphosphonate/vitamin D started. Cognition mildly abnormal but unchanged from baseline per family; no delirium.			
Relevant Investigations and Results Hb 108 (transfused 1U). Bone profile: Ca 2.28, vit D 32 (low). X-ray: satisfactory hemiarthroplasty position.			
Discharge Destination Community rehabilitation bed (Fleet Grange) for ongoing physiotherapy before planned return home.			
Relevant legal Information (e.g. was an independent Mental Capacity Act Advocate required) Mild cognitive impairment but retained capacity to consent to surgery/discharge planning; no DoLS or IMCA required. Family involved with patient's agreement.			
Information given to patient and/or authorised representative (including e.g. see GP in 2 weeks) Fracture, surgery and rehabilitation plan discussed with patient and son. Both counselled on hip precautions, falls-prevention at home, and bone-health medication compliance. Advised to seek urgent review for worsening pain, wound issues, fevers or new inability to weight-bear.			
Physical Ability & Cognitive Function :		On Admission	At Discharge
Physical	Unable to WB	Mobilising with elbow crutches/frame, physio ongoing	
Cognitive	Mild impaired	Unchanged from baseline; no delirium	
Other	Pain, deformed	Wound healing well, staples in situ for removal	
Advice, recommendations and future plans (including results awaited and outstanding investigations) G.P. Actions (Date) Arrange staple removal by 27/07/2026 if not done at rehab unit. For GP: continue bone protection (alendronic acid + AdCal D3) long-term; arrange DEXA if not already booked; falls clinic referral made. Counselling given as above re hip precautions, falls prevention and bone-health medication compliance. Strategies for potential problems Family/rehab staff aware of falls and cognitive history; escalation plan discussed with son.			

